

clinical-progress-note

Riverside Family Medicine — Office Visit Note

Patient DOB on file. Visit Date: 2026-07-14 **Provider:** Elena Marsh, MD

Subjective: Patient reports 4 months of low back pain radiating to the left posterior thigh and calf. Reports only about 20% improvement despite ongoing therapy (see attached PT summary).

Objective — Neurological Exam: Motor strength 4/5 left extensor hallucis longus (EHL) vs 5/5 right. Sensory exam shows decreased light touch in the left L5 dermatome. Deep tendon reflexes 2+ bilaterally at patellar, 1+ at left Achilles vs 2+ right. Straight-leg-raise test positive at 40 degrees on the left, negative on the right.

Red Flag Screen: No saddle anesthesia. No bowel or bladder dysfunction. No fever, chills, or history of IV drug use or recent spinal procedure. No unexplained weight loss or personal cancer history. No trauma history. Reviewed and negative for cauda equina syndrome, spinal infection, fracture, and malignancy.

Assessment: Lumbar radiculopathy, persistent despite conservative therapy.

Plan: MRI lumbar spine without contrast requested to evaluate for disc herniation or nerve root compression given the objective left L5 findings and lack of adequate response to conservative care, to guide next steps (possible epidural steroid injection vs. surgical referral). This imaging is expected to directly change management.

Electronically signed: Elena Marsh, MD